

[SYNTHETIC SAMPLE] Fictional record generated for the DELPHi prototype demo. Names, institutions, and statements are invented; the layout imitates a generic field-medical format. Not a real document and not derived from any real record.

[CONFERENCE REFERENCE] The meeting named above is a real, publicly announced scientific congress and is used here only as the setting of this fictional record. No session content, abstract, poster, or presented data from that meeting is reproduced or paraphrased; every person and conversation below is invented.

Congress Attendance Report — American Epilepsy Society Annual Meeting (AES) 2025

Meeting period: December 2025 | Author: Rachel Suh

Scope: unsolicited conversations held on-site. Session content and presented data are deliberately not reproduced here.

Conversation — Fiona Gallagher, MD (Great Lakes Epilepsy Institute)

I caught up with the physician between the Thursday afternoon sessions, and most of what we covered was teaching rather than anything clinical. The institute has taken on two additional fellows this year, so the supervision load has quietly grown without any corresponding change to clinic time. The plan for the fall is to restart a monthly journal club that had lapsed over the winter, ideally with the fellows selecting the papers themselves so that the discussion stays lively. I offered nothing further, since the conversation stayed entirely on training logistics.

Conversation — Victor Salas, MD (Hill Country Neuroscience Institute)

The Thursday session on drug-resistant focal epilepsy drew a full room, and this attending was first to the microphone once the floor opened. During Q&A, described a 17-year-old who failed two ASMs and asked, pointedly, when data below 18 would exist for cenobamate. I thanked the physician for raising it, confirmed that I am able to speak only to the approved adult indication for CNB, and made no comment on use in younger patients. The exchange has been logged for medical information f/u.

Conversation — Katherine Ost, MD (Sonoran Neuroscience Institute)

Our conversation at the Friday morning coffee break was almost entirely logistics. The physician is weighing whether to attend the regional epilepsy meeting in the spring, partly because the travel connections out of Tucson are awkward and partly because two colleagues from the same group have already committed. We compared notes on which sessions tend to fill up early, and I agreed to pass along the registration deadline once it is posted.

Conversation — Malika Whitmore, MD (Highmoor Health Neurology)

A short exchange over the coffee queue. Spent some time on the surgical workup list and how workup decisions get sequenced. Also mentioned one patient reporting fatigue that has not settled.

Conversation — Anouk Underhill, MD (Highmoor Health Neurology)

The conversation happened while waiting for a session room to clear. Agreed to reconnect at a follow-up appointment; nothing actionable this visit. Wants titration guidance written for caregivers rather than clinicians.

Conversation — Rosalind Dellacroix, MD (Lakeshore Neurological Care Center)

Stopped to talk on the way out of a session block, with no agenda on either side. Flagged, more than once, several patients whose seizures are generalized rather than focal, and therefore out of scope. Action item: references on the generalized tonic-clonic population, if available. The composition of newly referred patients has shifted over the last year or two.

Follow-ups have been logged in the interaction system.